

1. Administrative & Study Identification

Full Study Title: A Study to Evaluate and Compare the Efficacy and Safety of Alectinib Versus Crizotinib and to Evaluate the Pharmacokinetics of Alectinib in Asian Participants With Treatment-Naive Anaplastic Lymphoma Kinase (ALK)-Positive Advanced Non-Small Cell Lung Cancer (NSCLC) **Short Title/Acronym:** ALESIA **Protocol Number:** Y029449 **NCT Number:** NCT02838420 **Posting ID:** 1122466934205956783 **Trial Status:** Active, Not Recruiting (ACTIVE_NOT_RECRUITING) **Sponsor Name:** F. Hoffmann-La Roche Ltd (Roche) **Investigational Product:** Alectinib (Alecensa / R05424802) **Phase of Development:** Phase III **Medical Monitor:** Roche Clinical Operations

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To reliably determine whether the clinical benefit (in terms of investigator-assessed Progression-Free Survival [PFS]) of administering alectinib as a first-line treatment is consistent with the benefit observed in the global pivotal study ALEX (B028984). **Secondary Objectives:** To evaluate Independent Review Committee (IRC)-assessed PFS, Overall Survival (OS), Objective Response Rate (ORR), Duration of Response (DoR), time to central nervous system (CNS) progression, CNS objective response, and overall safety and pharmacokinetics.

2.2 Background & Rationale To address the clinical need for effective, targeted therapies in Asian populations with treatment-naïve ALK-positive advanced or metastatic non-small cell lung cancer (NSCLC). This trial serves to confirm if the robust progression-free survival benefit and CNS efficacy demonstrated in the global ALEX study translate consistently to Asian patients receiving 600 mg of alectinib twice daily.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Crizotinib) **Blinding:** Open-label **Randomization:** 2:1 (Alectinib vs. Crizotinib), stratified by Eastern Cooperative Oncology Group Performance Status (ECOG PS) and presence of baseline CNS metastases.

3.2 Planned Enrollment & Duration Targeted Enrollment: 187 patients **Number of Sites:** 21 investigational sites across China, South Korea, and Thailand **Estimated Study Start:** August 2016

Estimated Primary Completion: May 2017 (End of initial enrollment phase)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Asian participants aged ≥ 18 years. 2. Treatment-naïve Anaplastic Lymphoma Kinase (ALK)-positive advanced, recurrent, or metastatic Non-Small Cell Lung Cancer (NSCLC), assessed by the Ventana immunohistochemistry (IHC) test at a designated central laboratory. 3. Measurable disease per Response Evaluation Criteria in Solid Tumors (RECIST) version 1.1 at baseline. 4. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0, 1, or 2. 5. Life expectancy of at least 12 weeks. 6. Adequate hematologic function: Platelet count $\geq 100 \times 10^9/L$; absolute neutrophil count (ANC) ≥ 1500 cells/ μL ; hemoglobin ≥ 9.0 g/dL. 7. Adequate renal function: estimated glomerular filtration rate (eGFR) ≥ 45 mL/min/1.73 m^2 . 8. Negative serum pregnancy test for females of childbearing potential within 3 days prior to treatment; agreement to use highly effective contraceptive methods for both men and women.

4.2 Exclusion Criteria 1. Prior systemic treatment for advanced/metastatic ALK-positive NSCLC. 2. Symptomatic central nervous system (CNS) metastases (participants with asymptomatic CNS metastases or those who have successfully completed whole-brain radiation/gamma knife treatment ≥ 14 days before enrollment are permitted). 3. Unacceptable hepatic organ function, acute viral/active autoimmune hepatitis, or malabsorption syndrome. 4. Baseline QTc > 470 ms or symptomatic bradycardia. 5. Pregnant, lactating, or known HIV/AIDS-related illness.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Alectinib 600 mg administered twice daily (BID). **Route of Administration:** Oral (capsules), taken with food. **Duration of Treatment:** Continued until disease progression, unacceptable toxicity, withdrawal of consent, or death. **Control Regimen:** Crizotinib (Trade Name: Xalkori; ATC Code: L01ED01) 250 mg administered orally BID, with or without food.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care; localized radiation for asymptomatic CNS lesions at investigator discretion. **Prohibited:** Other investigational agents or concurrent targeted anti-cancer therapies outside of the assigned study arms; strong CYP3A inhibitors/inducers or agents with QT-prolonging effects within 14 days prior to first dose.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Baseline Tumor Assessment (RECIST v1.1), Ventana ALK IHC Status Confirmation, ECG
Baseline	Day 0	Randomization (2:1), First Dose of Alectinib or Crizotinib
Treatment Period	Continuous	Safety Assessments, Pharmacokinetic Profiling, Regular Efficacy/Tumor Scans
End of Treatment	End of dosing	Safety Follow-up, Transition to Survival Follow-up
Survival Follow-up	Post-treatment	Ongoing OS Assessment

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-Free Survival (PFS), defined as the time from randomization to the first documentation of disease progression or death from any cause. **Measurement Method:** Investigator assessment using RECIST v1.1 criteria.

7.2 Secondary & Exploratory Endpoints - Independent Review Committee (IRC)-assessed PFS using RECIST v1.1. - Overall Survival (OS). - Percentage of Participants With Objective Response of Complete Response (CR) or Partial Response (PR) as Determined by Investigator Using RECIST v1.1. - Duration of Response (DoR). - Time to Progression of Disease in the CNS as Determined by IRC Using RECIST v1.1 and CNS Objective Response.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard continuous tracking of AEs, specifically evaluating Grade 3-5 adverse events and serious adverse events (SAEs) relative to the control arm. **Serious Adverse Events (SAEs):** Reported within standard protocol timelines (typically 24 hours of site awareness). **Data Monitoring Committee (DMC):** Independent review facility utilized for efficacy (IRC) and centralized safety assessments.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for all primary efficacy analyses; Safety Set (all patients who received at least one dose of study medication) for safety evaluations. **Sample Size Justification:** A total of 97 PFS events were required to achieve approximately 87% probability to demonstrate consistency with the ALEX study, based on an assumed PFS Hazard Ratio (HR) of 0.65. This necessitated a target enrollment of 187 patients randomized 2:1. **Handling of Missing Data:** Standard right-censoring methods applied for time-to-event efficacy

variables (PFS, OS, DoR) for patients without documented events at the data cutoff.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring to ensure protocol adherence and safety compliance. **Audit Trail:**

Independent imaging group for central review of RECIST v1.1 tumor assessments to ensure objective progression tracking.

11. Study Identifiers & Governance

Primary ID: Y029449 **Registry Number:** NCT02838420 **Posting ID:**

1122466934205956783 **Sponsor/Lead Organization:** F. Hoffmann-La

Roche Ltd (Roche) **Study Title:** ALESIA **Official Regulatory Title:**

RANDOMIZED, MULTICENTER, PHASE III, OPEN-LABEL STUDY OF ALECTINIB VERSUS CRIZOTINIB IN ASIAN PATIENTS WITH TREATMENT-NAIVE

ANAPLASTIC LYMPHOMA KINASE-POSITIVE ADVANCED NON-SMALL CELL LUNG CANCER

12. Clinical Framework (Oncology Specific)

Primary Condition: Anaplastic Lymphoma Kinase-positive Non-small Cell Lung Cancer **Preferred UMLS Name:** ALK-Positive Non-Small Cell Lung Cancer (Anaplastic lymphoma kinase fusion oncogene positive non-small cell lung cancer) **SNOMED ID:** 830151004 **Diagnosis**

Threshold: Histologically/cytologically confirmed advanced, recurrent, or metastatic NSCLC with ALK-positive mutation confirmed by central Ventana IHC testing. **Medical Methodology:**

Targeted Tyrosine Kinase Inhibitor (TKI) Therapy **Optimization**

Target: Disease progression control and CNS metastasis mitigation

13. Study Procedures & Methodology

Management Pathway: First-line TKI targeted treatment compared to previous standard of care (Crizotinib). **Education Protocol (HCP):**

Investigator training on RECIST v1.1 evaluation and ALK mutation testing standards. **Education Protocol (Patient):** Dosing adherence

(with food for Alectinib) and AE symptom self-reporting. **Quality**

Audit Mechanism: Independent Review Committee (IRC) radiological assessments.

14. Observation & Follow-Up Schedule

Total Duration: Follow-up extended until 55% of patients have died for final Overall Survival (OS) analysis (approx. 41 months after first patient enrolled). **Onsite Visit Cadence:** Regular clinical visits corresponding with continuous daily dosing schedules. **Remote Monitoring Frequency:** Follow-up survival calls standard for advanced oncology trials. **Checklist Review Interval:** Regular interval scans per RECIST v1.1.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead	
Statistician	[Name]	_____	[DD-MMM-YYYY]				